

SECTION 2: TUBERCULOSIS DIAGNOSTIC PROTOCOL

2.1 CLINICAL SUSPICION:

Suspect active pulmonary tuberculosis in any patient with:

- Persistent unexplained cough lasting $\geq$ 2 weeks
- Unexplained weight loss, night sweats, or low-grade evening fever
- Hemoptysis (coughing up blood)
- Chest radiography showing apical or upper lobe cavitary lesions or nodular infiltrates.

2.2 DIAGNOSTIC WORKUP:

1. Sputum Examination: Collect two sputum specimens (one spot, one early morning) for Acid-Fast Bacilli
2. Molecular Testing: GeneXpert MTB/RIF provides simultaneous detection of Mycobacterium tuberculosis
3. Culture Confirmation: Liquid culture (MGIT) is the gold standard for definitive diagnosis and comprehens

2.3 INFECTION PREVENTION & ISOLATION:

Patients with suspected or confirmed infectious pulmonary TB must be placed in a negative pressure airbo